SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Vinorelbine IV · non-small-cell lung cancer

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Non-small-cell lung cancer · IV monotherapy · NCCP 00232 · version 5 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Vinorelbine chemotherapy can help control non-small-cell lung cancer and its symptoms. The team weighs possible benefit against side effects, previous treatments and your priorities, and reviews whether to continue.

Schedule
30 mg/m² on Days 1 and 8 every 21 days. Consultant may consider 25 mg/m² for poor marrow reserve; the dose is individualised after toxicity. Repeated 21-day cycles until progression/unacceptable toxicity; team review determines continuation.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Vinorelbine
VINCA ALKALOID CHEMOTHERAPY

- ☐ Low blood counts, infection and bleeding
- ☐ Nausea, vomiting and fatigue
- ☐ Constipation or bowel obstruction
- ☐ Nerve effects and uncommon lung/heart toxicity
- ☐ Infusion-site injury

Discuss the disease-control aim, likely benefit and uncertainty, other lung-cancer treatments or symptom-focused care, and the burden of visits and tests. Review low blood counts and infection, nausea/vomiting, fatigue, constipation/ileus, nerve symptoms, hair changes and uncommon serious lung/heart/allergic effects. Discuss IV-site injury and the different IV and capsule doses; the product-specific infusion plan needs pharmacy confirmation. Discuss fertility preservation and pregnancy planning. Do not breastfeed. Confirm the exact product contraception advice: HPRA recommends contraception during treatment and for 7 months afterwards for women, and 4 months for men.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00232 v5.

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